Lakeview Imaging Centre

27 Hill View Road, Pune | Ph: 020-3333-0000 (synthetic)

Patient: Kabir Iyer Age/Sex: 58 / M Study No: SYN-RAD-6120

Referred by: Dr. V. Nair, MD (Pulmonology) Report date: 03/10/2026

HIGH RESOLUTION CT THORAX (PLAIN)

Technique:

Plain axial HRCT of the thorax, 1 mm sections, no contrast given.

Prone and expiratory images obtained as needed.

OBSERVATIONS:

Mediastinum:

Trachea is normal in size and position.

Cardiac silhouette is normal.

Great vessels are normal in calibre.

Enlarged right paratracheal lymph node, measuring 1.6 x 1.0 cm.

No other mediastinal or hilar lymph node enlargement.

A 0.5 cm hypodense lesion is seen in the left lobe of the thyroid.

Lungs and Pleura:

Emphysematous changes are seen in both upper lobes.

A 3 x 2 mm subpleural nodule is seen in the right upper lobe.

No consolidation or ground glass opacity.

Both main bronchi are patent.

No evidence of pleural effusion.

No pneumothorax.

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HIGH RESOLUTION CT THORAX (PLAIN)

IMPRESSION:

- Centrilobular emphysema, predominantly involving both upper lobes.
- Mediastinal lymphadenopathy (largest 1.6 x 1.0 cm, right paratracheal).
- Incidental 3 mm subpleural nodule in the right upper lobe.
- Hypodense lesion in the left thyroid lobe, likely colloid cyst.

RECOMMENDATION:

Follow-up HRCT in 6 months. Ultrasound of the thyroid is advised.

Dr. M. Fernandes
MD, Consultant Radiologist Reg. No. SYN-6006 (synthetic)